NURS 1017 Unit 7 Case Study 2

19-year-old male client who fell from a longboard without a helmet: concussion picture, then a right temporal epidural hematoma with a lucid interval and uncal herniation (Monro-Kellie doctrine, Cushing's triad), care after craniotomy to prevent secondary brain injury (ICP, CPP), and discharge teaching on TBI prevention and graded return to activity.

Readiness problems: none

NURS 1017 Unit 7 Case Study 2, screen 1

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes and the Vital Signs. **Highlight** the findings that require follow-up.

Nurses' Notes | **Vital Signs**

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

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NURS 1017 Unit 7 Case Study 2, screen 1 (answer key, score 6/6)

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes and the Vital Signs. **Highlight** the findings that require follow-up.

Nurses' Notes | **Vital Signs**

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

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NURS 1017 Unit 7 Case Study 2, screen 2

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department is caring for a 19-year-old male client who fell from a longboard and struck his head.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

1615: Neurosurgery consulted; plan is observation with neurological checks every 15 minutes and a repeat CT in 4 hours. Client states his headache is "a bit better" and asks to go home.

The nurse has reviewed the Diagnostic Results and the Nurses' Notes from 1615.

➤ The client asks how his injury differs from a concussion. For each finding, click to specify whether it is consistent with a **concussion**, an **epidural hematoma**, a **subdural hematoma**, or a **subarachnoid hemorrhage**. Each finding may support more than one condition.

Finding	Concussion	Epidural hematoma	Subdural hematoma	Subarachnoid hemorrhage
Bleeding from a torn middle meningeal artery, often with a temporal bone fracture	☐	☐	☐	☐
Brief loss of consciousness with amnesia for the event and a normal CT scan	☐	☐	☐	☐
Crescent-shaped collection from torn bridging veins that can spread across suture lines	☐	☐	☐	☐
Headache, nausea, and vomiting	☐	☐	☐	☐
Loss of consciousness, then a lucid interval, then rapid deterioration	☐	☐	☐	☐
Sudden, severe ("worst ever") headache with neck stiffness from blood in the space around the brain	☐	☐	☐	☐
Symptoms that may develop slowly over weeks in older adults and clients taking anticoagulants	☐	☐	☐	☐
Biconvex (lens-shaped) collection that stops at the suture lines	☐	☐	☐	☐

Note: Each column must have at least 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 2, screen 2 (answer key, score 11/11)

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department is caring for a 19-year-old male client who fell from a longboard and struck his head.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

1615: Neurosurgery consulted; plan is observation with neurological checks every 15 minutes and a repeat CT in 4 hours. Client states his headache is "a bit better" and asks to go home.

The nurse has reviewed the Diagnostic Results and the Nurses' Notes from 1615.

➤ The client asks how his injury differs from a concussion. For each finding, click to specify whether it is consistent with a **concussion**, an **epidural hematoma**, a **subdural hematoma**, or a **subarachnoid hemorrhage**. Each finding may support more than one condition.

Finding	Concussion	Epidural hematoma	Subdural hematoma	Subarachnoid hemorrhage
Bleeding from a torn middle meningeal artery, often with a temporal bone fracture	☐	☑	☐	☐
Brief loss of consciousness with amnesia for the event and a normal CT scan	☑	☐	☐	☐
Crescent-shaped collection from torn bridging veins that can spread across suture lines	☐	☐	☑	☐
Headache, nausea, and vomiting	☑	☑	☑	☑
Loss of consciousness, then a lucid interval, then rapid deterioration	☐	☑	☐	☐
Sudden, severe ("worst ever") headache with neck stiffness from blood in the space around the brain	☐	☐	☐	☑
Symptoms that may develop slowly over weeks in older adults and clients taking anticoagulants	☐	☐	☑	☐
Biconvex (lens-shaped) collection that stops at the suture lines	☐	☑	☐	☐

Note: Each column must have at least 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 2, screen 3

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department is caring for a 19-year-old male client who fell from a longboard and struck his head.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

1615: Neurosurgery consulted; plan is observation with neurological checks every 15 minutes and a repeat CT in 4 hours. Client states his headache is "a bit better" and asks to go home.

1720: Client difficult to rouse: opens eyes only to a painful stimulus, speaks in confused, inappropriate words, and localizes pain (GCS 10, E2 V3 M5). Vomited twice since 1700. Right pupil 6 mm and sluggish; left pupil 3 mm and brisk. Left arm and leg weaker than the right (3/5). Respirations irregular.

The nurse has reviewed the Nurses' Notes from 1720 and the Vital Signs from 1720.

➤ Complete the following sentences by choosing from the lists of options.

The client's deterioration is most likely caused by Select... .

Because the skull cannot expand, the added Select... volume was at first offset by moving Select... out of the skull. That compensation has now failed, and the rising ICP is causing

Select...

.

Submit

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NURS 1017 Unit 7 Case Study 2, screen 3 (answer key, score 4/4)

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department is caring for a 19-year-old male client who fell from a longboard and struck his head.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

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The nurse has reviewed the Nurses' Notes from 1720 and the Vital Signs from 1720.

➤ Complete the following sentences by choosing from the lists of options.

The client's deterioration is most likely caused by [expansion of the epidural hematoma] .

Because the skull cannot expand, the added [blood] volume was at first offset by moving [cerebrospinal fluid and venous blood] out of the skull. That compensation has now failed, and the rising ICP is causing

[herniation of the right temporal lobe (uncus) that compresses cranial nerve III and the brainstem] .

Submit

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NURS 1017 Unit 7 Case Study 2, screen 4

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department is caring for a 19-year-old male client who fell from a longboard and struck his head.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders**

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

1615: Neurosurgery consulted; plan is observation with neurological checks every 15 minutes and a repeat CT in 4 hours. Client states his headache is "a bit better" and asks to go home.

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2130: Returned from the operating room after emergency right temporal craniotomy and evacuation of the epidural hematoma (bleeding middle meningeal artery coagulated). Admitted to the neurological ICU. Intubated and mechanically ventilated; sedated with propofol. Intraparenchymal ICP monitor in place: ICP 12 mm Hg. Pupils 3 mm, equal and reactive.

The nurse has reviewed the Nurses' Notes from 2130 and the Provider Orders.

➤ The nurse plans care to prevent secondary brain injury. For each intervention, click to specify whether it is **indicated** or **contraindicated**.

Intervention	Indicated	Contraindicated
Keep the head and neck in midline alignment and avoid tight endotracheal tube ties around the neck	○	○
Hyperventilate the client routinely to keep the $PaCO_2$ near 25 mm Hg	○	○
Report a systolic BP below 110 mm Hg promptly	○	○
Position the client with the hips flexed more than 90 degrees	○	○
Change the maintenance IV fluid to 5% dextrose in water	○	○
Give the prescribed acetaminophen and use cooling measures for a temperature above 38° C	○	○
Hyperoxygenate before suctioning and limit each suction pass to 10 seconds or less	○	○
Group the bath, linen change, suctioning, and repositioning into one long session	○	○

Note: Each row must have only 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 2, screen 4 (answer key, score 8/8)

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department is caring for a 19-year-old male client who fell from a longboard and struck his head.

Nurses' Notes | Vital Signs | Diagnostic Results | Provider Orders

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

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The nurse has reviewed the Nurses' Notes from 2130 and the Provider Orders.

➤ The nurse plans care to prevent secondary brain injury. For each intervention, click to specify whether it is **indicated** or **contraindicated**.

Intervention	Indicated	Contraindicated
Keep the head and neck in midline alignment and avoid tight endotracheal tube ties around the neck	●	○
Hyperventilate the client routinely to keep the $PaCO_2$ near 25 mm Hg	○	●
Report a systolic BP below 110 mm Hg promptly	●	○
Position the client with the hips flexed more than 90 degrees	○	●
Change the maintenance IV fluid to 5% dextrose in water	○	●
Give the prescribed acetaminophen and use cooling measures for a temperature above 38° C	●	○
Hyperoxygenate before suctioning and limit each suction pass to 10 seconds or less	●	○
Group the bath, linen change, suctioning, and repositioning into one long session	○	●

Note: Each row must have only 1 response option selected.

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NURS 1017 Unit 7 Case Study 2, screen 5

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse in the emergency department is caring for a 19-year-old male client who fell from a longboard and struck his head.

Nurses' Notes | Vital Signs | Diagnostic Results | Provider Orders

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

1615: Neurosurgery consulted; plan is observation with neurological checks every 15 minutes and a repeat CT in 4 hours. Client states his headache is "a bit better" and asks to go home.

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2130: Returned from the operating room after emergency right temporal craniotomy and evacuation of the epidural hematoma (bleeding middle meningeal artery coagulated). Admitted to the neurological ICU. Intubated and mechanically ventilated; sedated with propofol. Intraparenchymal ICP monitor in place: ICP 12 mm Hg. Pupils 3 mm, equal and reactive.

Day 2 0400: Client coughing and pushing against the ventilator during repositioning; sedation scale shows light sedation. ICP 26 mm Hg for the last 10 minutes; MAP 78 mm Hg. Head turned to the right on the pillow. Skin warm and flushed.

The nurse has reviewed the Nurses' Notes from Day 2 0400 and the Vital Signs from Day 2 0400.

Which actions should the nurse take? **Select all that apply.**

- [ ] 1. Lower the head of the bed flat to increase blood flow to the brain
- [ ] 2. Complete the morning bed bath now while the client is awake
- [ ] 3. Administer the prescribed 3% sodium chloride
- [ ] 4. Notify the provider of the ICP and the CPP
- [ ] 5. Reposition the head to midline and confirm the head of the bed is at 30 degrees
- [ ] 6. Administer the prescribed PRN fentanyl
- [ ] 7. Give the prescribed acetaminophen and apply cooling measures
- [ ] 8. Suction the endotracheal tube every 15 minutes to prevent coughing

Submit

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NURS 1017 Unit 7 Case Study 2, screen 5 (answer key, score 5/5)

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse in the emergency department is caring for a 19-year-old male client who fell from a longboard and struck his head.

Nurses' Notes | Vital Signs | Diagnostic Results | Provider Orders

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

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2130: Returned from the operating room after emergency right temporal craniotomy and evacuation of the epidural hematoma (bleeding middle meningeal artery coagulated). Admitted to the neurological ICU. Intubated and mechanically ventilated; sedated with propofol. Intraparenchymal ICP monitor in place: ICP 12 mm Hg. Pupils 3 mm, equal and reactive.

Day 2 0400: Client coughing and pushing against the ventilator during repositioning; sedation scale shows light sedation. ICP 26 mm Hg for the last 10 minutes; MAP 78 mm Hg. Head turned to the right on the pillow. Skin warm and flushed.

The nurse has reviewed the Nurses' Notes from Day 2 0400 and the Vital Signs from Day 2 0400.

Which actions should the nurse take? **Select all that apply.**

- [ ] 1. Lower the head of the bed flat to increase blood flow to the brain
- [ ] 2. Complete the morning bed bath now while the client is awake
- [x] 3. Administer the prescribed 3% sodium chloride
- [x] 4. Notify the provider of the ICP and the CPP
- [x] 5. Reposition the head to midline and confirm the head of the bed is at 30 degrees
- [x] 6. Administer the prescribed PRN fentanyl
- [x] 7. Give the prescribed acetaminophen and apply cooling measures
- [ ] 8. Suction the endotracheal tube every 15 minutes to prevent coughing

Submit

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NURS 1017 Unit 7 Case Study 2, screen 6

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse in the emergency department is caring for a 19-year-old male client who fell from a longboard and struck his head.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders**

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

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Day 2 0400: Client coughing and pushing against the ventilator during repositioning; sedation scale shows light sedation. ICP 26 mm Hg for the last 10 minutes; MAP 78 mm Hg. Head turned to the right on the pillow. Skin warm and flushed.

Day 9 1000: Extubated on Day 4; transferred to the neurosurgical unit on Day 5. GCS 15. Mild headache (2/10), tires easily, and says bright screens bother him. Incision healing. Discharge home with his parents today; follow-up with neurosurgery in 2 weeks. The

The nurse has reviewed the Nurses' Notes from Day 9 1000 and the Vital Signs from Day 9 1000.

➤ For each statement the client makes, click to specify whether it indicates an **understanding** of the discharge teaching or a **need for further teaching**.

Client Statement	Understanding	Needs further teaching
"I'll wear a certified helmet every time I ride my longboard or bike."	○	○
"I can go back to playing hockey as soon as my headache is gone."	○	○
"A few beers at the party this weekend won't affect how my brain heals."	○	○
"I'll drive myself to my follow-up appointment next week."	○	○
"If my headache gets much worse, I keep vomiting, or I get very sleepy or confused, I'll go to emergency."	○	○
"I'll start with short periods of light activity and limit screen time, then increase slowly if my symptoms don't get worse."	○	○

Note: Each row must have only 1 response option selected.

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Finish attempt →

NURS 1017 Unit 7 Case Study 2, screen 6 (answer key, score 6/6)

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse in the emergency department is caring for a 19-year-old male client who fell from a longboard and struck his head.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders**

1540: Arrived by ambulance after falling from his longboard at about 1500 and striking the right side of his head on a curb. He was not wearing a helmet. His friend states he was "knocked out for about a minute", then woke up and was talking. Client is alert. Oriented to person and place but not to time; repeatedly asks "What happened?" and cannot remember the fall. Reports a headache rated 6/10 with nausea; vomited once in the ambulance. 4-cm boggy swelling with an abrasion over the right temporal area. No drainage from the nose or ears. Pupils equal, 3 mm, briskly reactive. Moves all extremities with equal strength. Glasgow Coma Scale (GCS) 14 (E4 V4 M6). Neck nontender with full range of motion; cervical collar removed by the ED physician. Plays recreational hockey; had a concussion last winter.

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The nurse has reviewed the Nurses' Notes from Day 9 1000 and the Vital Signs from Day 9 1000.

➤ For each statement the client makes, click to specify whether it indicates an **understanding** of the discharge teaching or a **need for further teaching**.

Client Statement	Understanding	Needs further teaching
"I'll wear a certified helmet every time I ride my longboard or bike."	●	○
"I can go back to playing hockey as soon as my headache is gone."	○	●
"A few beers at the party this weekend won't affect how my brain heals."	○	●
"I'll drive myself to my follow-up appointment next week."	○	●
"If my headache gets much worse, I keep vomiting, or I get very sleepy or confused, I'll go to emergency."	●	○
"I'll start with short periods of light activity and limit screen time, then increase slowly if my symptoms don't get worse."	●	○

Note: Each row must have only 1 response option selected.

Submit

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Finish attempt →

NURS 1017 Unit 7 Case Study 3

19-year-old university student with meningococcal meningitis and early sepsis: meningeal signs, bacterial vs viral CSF findings, increased ICP and the types of cerebral edema, droplet precautions and public health follow-up, a generalized tonic-clonic seizure (seizure classification), communicating hydrocephalus with an external ventricular drain, and evaluation of complications.

Readiness problems: none

NURS 1017 Unit 7 Case Study 3, screen 1

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | **Vital Signs**

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

The nurse reviews the client's history and assessment. Which of the following findings would be consistent with **bacterial meningitis**? **Select all that apply.**

- [ ] 1. Photophobia
- [ ] 2. Abdomen soft and nontender
- [ ] 3. Confusion and drowsiness
- [ ] 4. Neck stiffness with pain on flexion
- [ ] 5. Positive Kernig and Brudzinski signs
- [ ] 6. Temperature 39.6° C
- [ ] 7. Last menstrual period 2 weeks ago
- [ ] 8. SpO_2 96% on room air
- [ ] 9. Severe headache with vomiting
- [ ] 10. Pupils equal and reactive

Submit

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NURS 1017 Unit 7 Case Study 3, screen 1 (answer key, score 6/6)

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | **Vital Signs**

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

The nurse reviews the client's history and assessment. Which of the following findings would be consistent with **bacterial meningitis**? **Select all that apply.**

- [x] 1. Photophobia
- [ ] 2. Abdomen soft and nontender
- [x] 3. Confusion and drowsiness
- [x] 4. Neck stiffness with pain on flexion
- [x] 5. Positive Kernig and Brudzinski signs
- [x] 6. Temperature 39.6° C
- [ ] 7. Last menstrual period 2 weeks ago
- [ ] 8. SpO_2 96% on room air
- [x] 9. Severe headache with vomiting
- [ ] 10. Pupils equal and reactive

Submit

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NURS 1017 Unit 7 Case Study 3, screen 2

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Laboratory Results**

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

Day 1 2215: Two sets of blood cultures drawn. Dexamethasone 10 mg IV given at 2218, then ceftriaxone 2 g IV and vancomycin 1.25 g IV started at 2220. Droplet precautions in place.

Day 1 2310: Lumbar puncture done after the head CT; CSF sent to the laboratory.

The nurse has reviewed the Nurses' Notes from Day 1 2215 and 2310, the Diagnostic Results and the Laboratory Results.

➤ For each finding, click to specify whether it is consistent with **bacterial meningitis**, **viral meningitis**, or both. Each finding may support more than one condition.

Finding	Bacterial meningitis	Viral meningitis
Cloudy (turbid) CSF	☐	☐
CSF white blood cells mostly lymphocytes	☐	☐
Fever, headache, and neck stiffness	☐	☐
CSF glucose much lower than normal	☐	☐
Organisms seen on the CSF Gram stain	☐	☐
Clear CSF with a normal glucose	☐	☐
Elevated CSF protein	☐	☐
CSF white blood cells mostly neutrophils	☐	☐

Note: Each column must have at least 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 3, screen 2 (answer key, score 10/10)

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Laboratory Results**

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

Day 1 2215: Two sets of blood cultures drawn. Dexamethasone 10 mg IV given at 2218, then ceftriaxone 2 g IV and vancomycin 1.25 g IV started at 2220. Droplet precautions in place.

Day 1 2310: Lumbar puncture done after the head CT; CSF sent to the laboratory.

The nurse has reviewed the Nurses' Notes from Day 1 2215 and 2310, the Diagnostic Results and the Laboratory Results.

➤ For each finding, click to specify whether it is consistent with **bacterial meningitis**, **viral meningitis**, or both. Each finding may support more than one condition.

Finding	Bacterial meningitis	Viral meningitis
Cloudy (turbid) CSF	☑	☐
CSF white blood cells mostly lymphocytes	☐	☑
Fever, headache, and neck stiffness	☑	☑
CSF glucose much lower than normal	☑	☐
Organisms seen on the CSF Gram stain	☑	☐
Clear CSF with a normal glucose	☐	☑
Elevated CSF protein	☑	☑
CSF white blood cells mostly neutrophils	☑	☐

Note: Each column must have at least 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 3, screen 3

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | Vital Signs | Diagnostic Results | Laboratory Results

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

Day 1 2215: Two sets of blood cultures drawn. Dexamethasone 10 mg IV given at 2218, then ceftriaxone 2 g IV and vancomycin 1.25 g IV started at 2220. Droplet precautions in place.

Day 1 2310: Lumbar puncture done after the head CT; CSF sent to the laboratory.

Complete the following sentences by choosing from the lists of options.

The client is at highest risk for Select... as a result of Select... . She is also at risk for Select... , as shown by Select... .

Submit

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NURS 1017 Unit 7 Case Study 3, screen 3 (answer key, score 4/4)

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | Vital Signs | Diagnostic Results | Laboratory Results

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

Day 1 2215: Two sets of blood cultures drawn. Dexamethasone 10 mg IV given at 2218, then ceftriaxone 2 g IV and vancomycin 1.25 g IV started at 2220. Droplet precautions in place.

Day 1 2310: Lumbar puncture done after the head CT; CSF sent to the laboratory.

Complete the following sentences by choosing from the lists of options.

The client is at highest risk for [increased intracranial pressure] as a result of [cerebral edema from the inflammatory response]. She is also at risk for [septic shock and disseminated intravascular coagulation], as shown by [hypotension, a nonblanching rash, a high lactate, and a low platelet count].

Submit

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NURS 1017 Unit 7 Case Study 3, screen 4

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | Vital Signs | Diagnostic Results | Laboratory Results | Provider Orders

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

Day 1 2215: Two sets of blood cultures drawn. Dexamethasone 10 mg IV given at 2218, then ceftriaxone 2 g IV and vancomycin 1.25 g IV started at 2220. Droplet precautions in place.

Day 1 2310: Lumbar puncture done after the head CT; CSF sent to the laboratory.

The nurse has reviewed the Provider Orders.

➤ Which interventions should the nurse include in the client's plan of care? **Select all that apply.**

- [ ] 1. Maintain droplet precautions until she has received 24 hours of effective antibiotic therapy
- [ ] 2. Support the neck in flexion on two pillows to ease the neck pain
- [ ] 3. Keep the room quiet and dimly lit
- [ ] 4. Restrict fluids to 500 mL per day to prevent cerebral edema
- [ ] 5. Implement seizure precautions
- [ ] 6. Elevate the head of the bed to 30 degrees
- [ ] 7. Place her in a negative-pressure room and wear an N95 respirator for all care
- [ ] 8. Give the antibiotics on time to maintain therapeutic blood levels
- [ ] 9. Confirm that public health has been notified so close contacts can receive preventive antibiotics

Submit

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NURS 1017 Unit 7 Case Study 3, screen 4 (answer key, score 6/6)

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Laboratory Results** | **Provider Orders**

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

Day 1 2215: Two sets of blood cultures drawn. Dexamethasone 10 mg IV given at 2218, then ceftriaxone 2 g IV and vancomycin 1.25 g IV started at 2220. Droplet precautions in place.

Day 1 2310: Lumbar puncture done after the head CT; CSF sent to the laboratory.

The nurse has reviewed the Provider Orders.

Which interventions should the nurse include in the client's plan of care? **Select all that apply.**

- [x] 1. Maintain droplet precautions until she has received 24 hours of effective antibiotic therapy
- [ ] 2. Support the neck in flexion on two pillows to ease the neck pain
- [x] 3. Keep the room quiet and dimly lit
- [ ] 4. Restrict fluids to 500 mL per day to prevent cerebral edema
- [x] 5. Implement seizure precautions
- [x] 6. Elevate the head of the bed to 30 degrees
- [ ] 7. Place her in a negative-pressure room and wear an N95 respirator for all care
- [x] 8. Give the antibiotics on time to maintain therapeutic blood levels
- [x] 9. Confirm that public health has been notified so close contacts can receive preventive antibiotics

Submit

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NOT COMPLETE

Case Study Screen 5 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Laboratory Results** | **Provider Orders**

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

Day 1 2215: Two sets of blood cultures drawn. Dexamethasone 10 mg IV given at 2218, then ceftriaxone 2 g IV and vancomycin 1.25 g IV started at 2220. Droplet precautions in place.

Day 1 2310: Lumbar puncture done after the head CT; CSF sent to the laboratory.

Day 2 0545: Client had a generalized tonic-clonic seizure lasting 2 minutes: sudden stiffening of the whole body, then rhythmic jerking of all four limbs; no warning or focal movements beforehand. Now postictal: snoring respirations, opens eyes to pain only (GCS 8). Reports of worsening headache and vomiting overnight. Pupils equal, 4 mm, sluggish.

The nurse has reviewed the Nurses' Notes from Day 2 0545, the Vital Signs from Day 2 0600 and the Provider Orders from Day 2 0550.

➤ For each action, click to specify whether it is **indicated** or **contraindicated** for the client at this time.

Nursing Action	Indicated	Contraindicated
Turn the client onto her side and suction the mouth as needed	○	○
Place a padded tongue blade between her teeth in case another seizure occurs	○	○
Apply oxygen and monitor her airway and breathing closely	○	○
Administer the levetiracetam loading dose	○	○
Prepare her for a repeat lumbar puncture to drain CSF and lower the pressure	○	○
Discontinue droplet precautions because she has received 8 hours of antibiotics	○	○
Prepare her for the STAT head CT	○	○

Note: Each row must have only 1 response option selected.

Submit

NURS 1017 Unit 7 Case Study 3, screen 5 (answer key, score 7/7)

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Laboratory Results** | **Provider Orders**

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

Day 1 2215: Two sets of blood cultures drawn. Dexamethasone 10 mg IV given at 2218, then ceftriaxone 2 g IV and vancomycin 1.25 g IV started at 2220. Droplet precautions in place.

Day 1 2310: Lumbar puncture done after the head CT; CSF sent to the laboratory.

Day 2 0545: Client had a generalized tonic-clonic seizure lasting 2 minutes: sudden stiffening of the whole body, then rhythmic jerking of all four limbs; no warning or focal movements beforehand. Now postictal: snoring respirations, opens eyes to pain only (GCS 8). Reports of worsening headache and vomiting overnight. Pupils equal, 4 mm, sluggish.

The nurse has reviewed the Nurses' Notes from Day 2 0545, the Vital Signs from Day 2 0600 and the Provider Orders from Day 2 0550.

➤ For each action, click to specify whether it is **indicated** or **contraindicated** for the client at this time.

Nursing Action	Indicated	Contraindicated
Turn the client onto her side and suction the mouth as needed	●	○
Place a padded tongue blade between her teeth in case another seizure occurs	○	●
Apply oxygen and monitor her airway and breathing closely	●	○
Administer the levetiracetam loading dose	●	○
Prepare her for a repeat lumbar puncture to drain CSF and lower the pressure	○	●
Discontinue droplet precautions because she has received 8 hours of antibiotics	○	●
Prepare her for the STAT head CT	●	○

Note: Each row must have only 1 response option selected.

Submit

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NOT COMPLETE

Case Study Screen 6 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Laboratory Results** | **Provider Orders**

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

Day 1 2215: Two sets of blood cultures drawn. Dexamethasone 10 mg IV given at 2218, then ceftriaxone 2 g IV and vancomycin 1.25 g IV started at 2220. Droplet precautions in place.

Day 1 2310: Lumbar puncture done after the head CT; CSF sent to the laboratory.

Day 2 0545: Client had a generalized tonic-clonic seizure lasting 2 minutes: sudden stiffening of the whole body, then rhythmic jerking of all four limbs; no warning or focal movements beforehand. Now postictal: snoring respirations, opens eyes to pain only (GCS 8). Reports of worsening headache and vomiting overnight. Pupils equal, 4 mm, sluggish.

Day 2 0730: External ventricular drain (EVD) inserted by neurosurgery; opening ICP 28 mm Hg. Draining clear, slightly cloudy CSF.

Day 12 1000: EVD removed on Day 8; repeat CT shows normal ventricles. Client alert and oriented, ambulating. No seizures since Day 2. Reports she "can't hear well" in her right ear. Dark, dry areas of skin on two toes of the left foot. Roommates and her boyfriend received ciprofloxacin on Day 2. Discharge planned for Day 14.

The nurse has reviewed the Diagnostic Results from Day 2, the Nurses' Notes from Day 2 0730 and Day 12 1000, and the Vital Signs and Laboratory Results from Day 12.

➤ For each finding, click to specify whether it shows that an **expected outcome has been met** or that it **requires follow-up**.

Finding	Expected outcome met	Requires follow-up
Alert and oriented, with no seizures since Day 2	○	○
Reduced hearing in the right ear	○	○
Normal ventricles on CT after removal of the drain	○	○
Dark, dry areas of skin on two toes	○	○
Platelet count 214 × 10^9/L and lactate 1.1 mmol/L	○	○
Close contacts received preventive antibiotics	○	○
Temperature 36.9° C and WBC 8.1 × 10^9/L	○	○

Note: Each row must have only 1 response option selected.

Submit

NURS 1017 Unit 7 Case Study 3, screen 6 (answer key, score 7/7)

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse in the emergency department is caring for a 19-year-old female university student brought in by her roommate.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Laboratory Results** | **Provider Orders**

Day 1 2200: Roommate states the client has had a fever, a severe headache and vomiting since this morning, and has become more confused over the last 2 hours. Client says, "My neck hurts when I bend it" and covers her eyes because the room lights hurt. Lives in a university residence; a friend on her floor had a "bad cold" last week. Unsure whether she received the school meningococcal vaccine. Drowsy, opens eyes to voice, oriented to person only (GCS 13: E3 V4 M6). Neck stiff with pain on flexion; positive Kernig and Brudzinski signs. Nonblanching petechial rash on the legs and trunk. Hands and feet cool; capillary refill 4 seconds. Pupils equal, 4 mm, reactive. Lungs clear. Abdomen soft, nontender. Last menstrual period 2 weeks ago.

Day 1 2215: Two sets of blood cultures drawn. Dexamethasone 10 mg IV given at 2218, then ceftriaxone 2 g IV and vancomycin 1.25 g IV started at 2220. Droplet precautions in place.

Day 1 2310: Lumbar puncture done after the head CT; CSF sent to the laboratory.

Day 2 0545: Client had a generalized tonic-clonic seizure lasting 2 minutes: sudden stiffening of the whole body, then rhythmic jerking of all four limbs; no warning or focal movements beforehand. Now postictal: snoring respirations, opens eyes to pain only (GCS 8). Reports of worsening headache and vomiting overnight. Pupils equal, 4 mm, sluggish.

Day 2 0730: External ventricular drain (EVD) inserted by neurosurgery; opening ICP 28 mm Hg. Draining clear, slightly cloudy CSF.

Day 12 1000: EVD removed on Day 8; repeat CT shows normal ventricles. Client alert and oriented, ambulating. No seizures since Day 2. Reports she "can't hear well" in her right ear. Dark, dry areas of skin on two toes of the left foot. Roommates and her boyfriend received ciprofloxacin on Day 2. Discharge planned for Day 14.

The nurse has reviewed the Diagnostic Results from Day 2, the Nurses' Notes from Day 2 0730 and Day 12 1000, and the Vital Signs and Laboratory Results from Day 12.

➤ For each finding, click to specify whether it shows that an **expected outcome has been met** or that it **requires follow-up**.

Finding	Expected outcome met	Requires follow-up
Alert and oriented, with no seizures since Day 2	●	○
Reduced hearing in the right ear	○	●
Normal ventricles on CT after removal of the drain	●	○
Dark, dry areas of skin on two toes	○	●
Platelet count 214 × 10^9/L and lactate 1.1 mmol/L	●	○
Close contacts received preventive antibiotics	●	○
Temperature 36.9° C and WBC 8.1 × 10^9/L	●	○

Note: Each row must have only 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 4

57-year-old female client with a first seizure in adulthood from a left frontotemporal glioblastoma: signs of increased ICP, glioblastoma vs meningioma vs brain metastasis, focal impaired awareness to bilateral tonic-clonic seizure, vasogenic vs cytotoxic edema, Monro-Kellie components and hydrocephalus types, status epilepticus after craniotomy, and corticosteroid complications at discharge.

Readiness problems: none

NURS 1017 Unit 7 Case Study 4, screen 1

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes and the Vital Signs. **Highlight** the findings that suggest increased intracranial pressure (ICP).

Nurses' Notes | **Vital Signs**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

Submit

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NURS 1017 Unit 7 Case Study 4, screen 1 (answer key, score 3/3)

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes and the Vital Signs. **Highlight** the findings that suggest increased intracranial pressure (ICP).

Nurses' Notes | **Vital Signs**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

Submit

← Quit

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NURS 1017 Unit 7 Case Study 4, screen 2

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department is caring for a 57-year-old female client brought in by her husband after a seizure at home.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

The nurse has reviewed the Diagnostic Results.

➤ The client's husband asks what kind of tumor this might be. For each feature, click to specify whether it is typical of a **glioblastoma**, a **meningioma**, or a **brain metastasis**. Each feature may apply to more than one tumor.

Feature	Glioblastoma	Meningioma	Brain metastasis
Arises from the meninges (arachnoid cells), outside the brain tissue	☐	☐	☐
Arises from glial cells (astrocytes) within the brain tissue	☐	☐	☐
Spreads to the brain through the blood from cancers such as lung, breast, or melanoma	☐	☐	☐
Can cause seizures, headache, and focal neurological deficits	☐	☐	☐
Usually benign and slow-growing; often cured by complete removal	☐	☐	☐
Ring-enhancing lesion with surrounding edema on MRI	☐	☐	☐
Infiltrates the surrounding brain, so complete removal is rarely possible	☐	☐	☐
Most common malignant primary brain tumor in adults	☐	☐	☐

Note: Each column must have at least 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 4, screen 2 (answer key, score 11/11)

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department is caring for a 57-year-old female client brought in by her husband after a seizure at home.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

The nurse has reviewed the Diagnostic Results.

➤ The client's husband asks what kind of tumor this might be. For each feature, click to specify whether it is typical of a **glioblastoma**, a **meningioma**, or a **brain metastasis**. Each feature may apply to more than one tumor.

Feature	Glioblastoma	Meningioma	Brain metastasis
Arises from the meninges (arachnoid cells), outside the brain tissue	☐	☑	☐
Arises from glial cells (astrocytes) within the brain tissue	☑	☐	☐
Spreads to the brain through the blood from cancers such as lung, breast, or melanoma	☐	☐	☑
Can cause seizures, headache, and focal neurological deficits	☑	☑	☑
Usually benign and slow-growing; often cured by complete removal	☐	☑	☐
Ring-enhancing lesion with surrounding edema on MRI	☑	☐	☑
Infiltrates the surrounding brain, so complete removal is rarely possible	☑	☐	☐
Most common malignant primary brain tumor in adults	☑	☐	☐

Note: Each column must have at least 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 4, screen 3

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department is caring for a 57-year-old female client brought in by her husband after a seizure at home.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

➤ Complete the following sentences by choosing from the lists of options.

The seizure the husband described is best classified as a Select... . The client's signs of increased ICP are mainly caused by Select... , which results from Select... and is expected to improve with Select... .

Submit

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NURS 1017 Unit 7 Case Study 4, screen 3 (answer key, score 4/4)

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department is caring for a 57-year-old female client brought in by her husband after a seizure at home.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

➤ Complete the following sentences by choosing from the lists of options.

The seizure the husband described is best classified as a

[focal impaired awareness seizure that progressed to a bilateral tonic-clonic seizure] . The client's signs of increased ICP are mainly caused by

[vasogenic edema] , which results from

[abnormal tumor blood vessels that let plasma leak through a damaged blood-brain barrier]

and is expected to improve with [dexamethasone] .

Submit

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NURS 1017 Unit 7 Case Study 4, screen 4

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department is caring for a 57-year-old female client brought in by her husband after a seizure at home.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

The nurse has reviewed the Provider Orders.

➤ The nurse plans care to keep the client's ICP from rising further. For each factor, click to specify which intracranial volume it increases: **brain tissue**, **blood**, or **cerebrospinal fluid (CSF)**.

Factor	Brain tissue	Blood	CSF
Vasogenic edema around the tumor	○	○	○
Hypoventilation from oversedation, raising the $PaCO_2$	○	○	○
A tumor blocking the cerebral aqueduct (non-communicating hydrocephalus)	○	○	○
Neck flexion or rotation that compresses the jugular veins	○	○	○
Cytotoxic edema after a prolonged seizure with hypoxia	○	○	○
Scarring of the arachnoid villi that impairs CSF absorption (communicating hydrocephalus)	○	○	○
Fever, which increases brain metabolism	○	○	○

Note: Each row must have only 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 4, screen 4 (answer key, score 7/7)

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department is caring for a 57-year-old female client brought in by her husband after a seizure at home.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

The nurse has reviewed the Provider Orders.

➤ The nurse plans care to keep the client's ICP from rising further. For each factor, click to specify which intracranial volume it increases: **brain tissue**, **blood**, or **cerebrospinal fluid (CSF)**.

Factor	Brain tissue	Blood	CSF
Vasogenic edema around the tumor	●	○	○
Hypoventilation from oversedation, raising the $PaCO_2$	○	●	○
A tumor blocking the cerebral aqueduct (non-communicating hydrocephalus)	○	○	●
Neck flexion or rotation that compresses the jugular veins	○	●	○
Cytotoxic edema after a prolonged seizure with hypoxia	●	○	○
Scarring of the arachnoid villi that impairs CSF absorption (communicating hydrocephalus)	○	○	●
Fever, which increases brain metabolism	○	●	○

Note: Each row must have only 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 4, screen 5

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse in the emergency department is caring for a 57-year-old female client brought in by her husband after a seizure at home.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

Day 3 1500: Returned from craniotomy and subtotal resection of the left frontotemporal tumor. Awake, oriented, mild word-finding difficulty unchanged from before surgery. Dressing dry.

Day 4 0710: Called to the room by a nursing assistant: client is having a generalized tonic-clonic seizure that started at 0705 and is still continuing. Lips dusky; frothy saliva. Side rails up and padded; IV saline lock patent.

The nurse has reviewed the Nurses' Notes from Day 3 1500 and Day 4 0710 and the Provider Orders from Day 3 1500.

Which actions should the nurse take now? **Select all that apply.**

- [ ] 1. Call for help and notify the provider
- [ ] 2. Apply oxygen and have suction ready
- [ ] 3. Withhold medication until the seizure has lasted 15 minutes
- [ ] 4. Insert a padded tongue blade between her teeth
- [ ] 5. Administer the prescribed IV lorazepam
- [ ] 6. Check her capillary blood glucose
- [ ] 7. Turn the client onto her side
- [ ] 8. Restrain her arms and legs to prevent injury

Submit

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NURS 1017 Unit 7 Case Study 4, screen 5 (answer key, score 5/5)

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse in the emergency department is caring for a 57-year-old female client brought in by her husband after a seizure at home.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

Day 3 1500: Returned from craniotomy and subtotal resection of the left frontotemporal tumor. Awake, oriented, mild word-finding difficulty unchanged from before surgery. Dressing dry.

Day 4 0710: Called to the room by a nursing assistant: client is having a generalized tonic-clonic seizure that started at 0705 and is still continuing. Lips dusky; frothy saliva. Side rails up and padded; IV saline lock patent.

The nurse has reviewed the Nurses' Notes from Day 3 1500 and Day 4 0710 and the Provider Orders from Day 3 1500.

Which actions should the nurse take now? **Select all that apply.**

- [x] 1. Call for help and notify the provider
- [x] 2. Apply oxygen and have suction ready
- [ ] 3. Withhold medication until the seizure has lasted 15 minutes
- [ ] 4. Insert a padded tongue blade between her teeth
- [x] 5. Administer the prescribed IV lorazepam
- [x] 6. Check her capillary blood glucose
- [x] 7. Turn the client onto her side
- [ ] 8. Restrain her arms and legs to prevent injury

Submit

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Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse in the emergency department is caring for a 57-year-old female client brought in by her husband after a seizure at home.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders** | **Laboratory Results**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

Day 3 1500: Returned from craniotomy and subtotal resection of the left frontotemporal tumor. Awake, oriented, mild word-finding difficulty unchanged from before surgery. Dressing dry.

Day 4 0710: Called to the room by a nursing assistant: client is having a generalized tonic-clonic seizure that started at 0705 and is still continuing. Lips dusky; frothy saliva. Side rails up and padded; IV saline lock patent.

Day 9 1000: No further seizures since Day 4 (levetiracetam increased to 1,000 mg every 12 hours). Headache 2/10, no vomiting. Incision clean and dry, staples intact. White patches on the tongue and inner cheeks that do not wipe off easily. Dexamethasone now 4 mg PO twice daily with a planned taper. Referred to the cancer centre for radiation and temozolomide. The nurse reviews the discharge teaching.

Day 9 1030: Client states: "I'll stop the dexamethasone once my headaches are gone." Also states: "I won't drive until my doctor tells me it's safe." Husband states: "If she has a seizure, I'll turn her on her side, time it, and call 911 if it lasts 5 minutes."

The nurse has reviewed the Diagnostic Results from Day 5, the Nurses' Notes from Day 9 1000 and 1030, and the Laboratory Results.

➤ For each finding, click to specify whether it shows that an **expected outcome has been met** or that it **requires follow-up**.

Finding	Expected outcome met	Requires follow-up
No seizures since Day 4	○	○
Capillary glucose 14.8 mmol/L before supper	○	○
Headache 2/10 with no vomiting	○	○
"I'll stop the dexamethasone once my headaches are gone."	○	○
White patches on the tongue and inner cheeks	○	○
"I won't drive until my doctor tells me it's safe."	○	○
Husband's plan for managing a seizure at home	○	○
Incision clean and dry, staples intact	○	○

Note: Each row must have only 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 4, screen 6 (answer key, score 8/8)

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse in the emergency department is caring for a 57-year-old female client brought in by her husband after a seizure at home.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders** | **Laboratory Results**

1910: Husband states they were watching television when she suddenly stopped talking and stared straight ahead, smacking her lips, with her right hand picking at her shirt; she did not answer him. About a minute later her whole body stiffened and then jerked for about 2 minutes. Bit her tongue; incontinent of urine. Over the past 6 weeks she has had headaches that are worst when she wakes up and when she coughs or bends over, and she vomited on waking twice this week without feeling nauseated. He says she has been "irritable and forgetful, not herself," and has trouble finding words. Now drowsy but rousable, oriented to person and place, slow to answer. Right hand grip slightly weaker than the left (4/5). Fundoscopic examination by the ED physician: bilateral papilledema. Point-of-care glucose 6.1 mmol/L.

Day 3 1500: Returned from craniotomy and subtotal resection of the left frontotemporal tumor. Awake, oriented, mild word-finding difficulty unchanged from before surgery. Dressing dry.

Day 4 0710: Called to the room by a nursing assistant: client is having a generalized tonic-clonic seizure that started at 0705 and is still continuing. Lips dusky; frothy saliva. Side rails up and padded; IV saline lock patent.

Day 9 1000: No further seizures since Day 4 (levetiracetam increased to 1,000 mg every 12 hours). Headache 2/10, no vomiting. Incision clean and dry, staples intact. White patches on the tongue and inner cheeks that do not wipe off easily. Dexamethasone now 4 mg PO twice daily with a planned taper. Referred to the cancer centre for radiation and temozolomide. The nurse reviews the discharge teaching.

Day 9 1030: Client states: "I'll stop the dexamethasone once my headaches are gone." Also states: "I won't drive until my doctor tells me it's safe." Husband states: "If she has a seizure, I'll turn her on her side, time it, and call 911 if it lasts 5 minutes."

The nurse has reviewed the Diagnostic Results from Day 5, the Nurses' Notes from Day 9 1000 and 1030, and the Laboratory Results.

➤ For each finding, click to specify whether it shows that an **expected outcome has been met** or that it **requires follow-up**.

Finding	Expected outcome met	Requires follow-up
No seizures since Day 4	●	○
Capillary glucose 14.8 mmol/L before supper	○	●
Headache 2/10 with no vomiting	●	○
"I'll stop the dexamethasone once my headaches are gone."	○	●
White patches on the tongue and inner cheeks	○	●
"I won't drive until my doctor tells me it's safe."	●	○
Husband's plan for managing a seizure at home	●	○
Incision clean and dry, staples intact	●	○

Note: Each row must have only 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 5

22-year-old male client with a complete C6 spinal cord injury after diving into shallow water: neurogenic vs spinal vs hypovolemic shock, respiratory risk (phrenic nerve, vital capacity), acute care after fusion, autonomic dysreflexia in rehabilitation, and teaching on prevention and lifelong self-care.

Readiness problems: none

NURS 1017 Unit 7 Case Study 5, screen 1

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes and the Vital Signs. **Highlight** the findings that require immediate follow-up.

Nurses' Notes | **Vital Signs**

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

Submit

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NURS 1017 Unit 7 Case Study 5, screen 1 (answer key, score 7/7)

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes and the Vital Signs. **Highlight** the findings that require immediate follow-up.

Nurses' Notes | **Vital Signs**

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

Submit

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NURS 1017 Unit 7 Case Study 5, screen 2

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department is caring for a 22-year-old male client who dove into shallow water at a lake.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Laboratory Results**

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

The nurse has reviewed the Diagnostic Results and the Laboratory Results.

➤ For each finding, click to specify whether it is consistent with **spinal shock**, **neurogenic shock**, or **hypovolemic shock**. Each finding may support more than one condition.

Finding	Spinal shock	Neurogenic shock	Hypovolemic shock
Flaccid paralysis and absent reflexes below the level of injury	☐	☐	☐
Hypotension	☐	☐	☐
Bradycardia	☐	☐	☐
Cool, clammy skin	☐	☐	☐
Loss of sympathetic nervous system outflow from an injury at T6 or above	☐	☐	☐
Warm, dry skin below the injury	☐	☐	☐
Tachycardia	☐	☐	☐
Loss of bladder and bowel reflexes, including anal sphincter tone	☐	☐	☐

Note: Each column must have at least 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 5, screen 2 (answer key, score 9/9)

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department is caring for a 22-year-old male client who dove into shallow water at a lake.

Nurses' Notes | Vital Signs | Diagnostic Results | Laboratory Results

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

The nurse has reviewed the Diagnostic Results and the Laboratory Results.

➤ For each finding, click to specify whether it is consistent with **spinal shock**, **neurogenic shock**, or **hypovolemic shock**. Each finding may support more than one condition.

Finding	Spinal shock	Neurogenic shock	Hypovolemic shock
Flaccid paralysis and absent reflexes below the level of injury	☑	☐	☐
Hypotension	☐	☑	☑
Bradycardia	☐	☑	☐
Cool, clammy skin	☐	☐	☑
Loss of sympathetic nervous system outflow from an injury at T6 or above	☐	☑	☐
Warm, dry skin below the injury	☐	☑	☐
Tachycardia	☐	☐	☑
Loss of bladder and bowel reflexes, including anal sphincter tone	☑	☐	☐

Note: Each column must have at least 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 5, screen 3

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department is caring for a 22-year-old male client who dove into shallow water at a lake.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Laboratory Results**

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

Day 1 1630: Vital capacity 1.1 L (predicted 4.8 L). Using neck muscles to breathe; says he feels "tired of breathing." Oxygen 4 L/min by nasal cannula applied.

The nurse has reviewed the Nurses' Notes from Day 1 1630 and the Vital Signs from Day 1 1630.

➤ Complete the following sentences by choosing from the lists of options.

The client's priority problem is Select... . His injury spares Select... but paralyzes Select... . His breathing may get worse over the next few days because Select... .

Submit

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NURS 1017 Unit 7 Case Study 5, screen 3 (answer key, score 4/4)

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department is caring for a 22-year-old male client who dove into shallow water at a lake.

Nurses' Notes | Vital Signs | Diagnostic Results | Laboratory Results

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

Day 1 1630: Vital capacity 1.1 L (predicted 4.8 L). Using neck muscles to breathe; says he feels "tired of breathing." Oxygen 4 L/min by nasal cannula applied.

The nurse has reviewed the Nurses' Notes from Day 1 1630 and the Vital Signs from Day 1 1630.

➤ Complete the following sentences by choosing from the lists of options.

The client's priority problem is [risk for respiratory failure] . His injury spares [the phrenic nerve (C3–C5), which controls the diaphragm] but paralyzes [the intercostal and abdominal muscles] . His breathing may get worse over the next few days because [edema of the spinal cord can extend upward toward C3–C5] .

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NURS 1017 Unit 7 Case Study 5, screen 4

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

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Case Study Screen 4 of 6

The nurse in the emergency department is caring for a 22-year-old male client who dove into shallow water at a lake.

Nurses' Notes | Vital Signs | Diagnostic Results | Laboratory Results | Provider Orders

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

Day 1 1630: Vital capacity 1.1 L (predicted 4.8 L). Using neck muscles to breathe; says he feels "tired of breathing." Oxygen 4 L/min by nasal cannula applied.

Day 2 0800: Underwent anterior cervical decompression and fusion of C5–C6 overnight; in a hard cervical collar. Remains on 4 L/min oxygen; vital capacity 1.3 L. Admitted to the ICU.

The nurse has reviewed the Nurses' Notes from Day 2 0800 and the Provider Orders.

➤ For each intervention, click to specify whether it is **indicated** or **contraindicated** for the client.

Intervention	Indicated	Contraindicated
Logroll the client with a second nurse holding the head in line with the body	○	○
Suction the airway for longer than 15 seconds to clear thick secretions	○	○
Measure the vital capacity and assess cough strength as ordered	○	○
Raise the head of the bed quickly to 90 degrees to ease his breathing	○	○
Titrate the norepinephrine to keep the MAP at 85–90 mm Hg	○	○
Remove the cervical collar to wash his neck while he lies alone	○	○
Turn him at least every 2 hours and inspect the skin over bony prominences	○	○
Monitor urine output through the indwelling catheter	○	○

Note: Each row must have only 1 response option selected.

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NURS 1017 Unit 7 Case Study 5, screen 4 (answer key, score 8/8)

Practice NCLEX RN Test

Question 4 of 6

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Case Study Screen 4 of 6

The nurse in the emergency department is caring for a 22-year-old male client who dove into shallow water at a lake.

Nurses' Notes | Vital Signs | Diagnostic Results | Laboratory Results | Provider Orders

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

Day 1 1630: Vital capacity 1.1 L (predicted 4.8 L). Using neck muscles to breathe; says he feels "tired of breathing." Oxygen 4 L/min by nasal cannula applied.

Day 2 0800: Underwent anterior cervical decompression and fusion of C5–C6 overnight; in a hard cervical collar. Remains on 4 L/min oxygen; vital capacity 1.3 L. Admitted to the ICU.

The nurse has reviewed the Nurses' Notes from Day 2 0800 and the Provider Orders.

➤ For each intervention, click to specify whether it is **indicated** or **contraindicated** for the client.

Intervention	Indicated	Contraindicated
Logroll the client with a second nurse holding the head in line with the body	●	○
Suction the airway for longer than 15 seconds to clear thick secretions	○	●
Measure the vital capacity and assess cough strength as ordered	●	○
Raise the head of the bed quickly to 90 degrees to ease his breathing	○	●
Titrate the norepinephrine to keep the MAP at 85–90 mm Hg	●	○
Remove the cervical collar to wash his neck while he lies alone	○	●
Turn him at least every 2 hours and inspect the skin over bony prominences	●	○
Monitor urine output through the indwelling catheter	●	○

Note: Each row must have only 1 response option selected.

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NURS 1017 Unit 7 Case Study 5, screen 5

Practice NCLEX RN Test

Question 5 of 6

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Case Study Screen 5 of 6

The nurse in the emergency department is caring for a 22-year-old male client who dove into shallow water at a lake.

Nurses' Notes | Vital Signs | Diagnostic Results | Laboratory Results | Provider Orders

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

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Day 2 0800: Underwent anterior cervical decompression and fusion of C5–C6 overnight; in a hard cervical collar. Remains on 4 L/min oxygen; vital capacity 1.3 L. Admitted to the ICU.

Week 5 1400: Rehabilitation unit. Spinal shock resolved in Week 3; reflexes and spasticity in the legs have returned. Client lying in bed, reports a sudden pounding headache and blurred vision. Face and neck flushed and sweating; skin below the clavicles pale and cool with goosebumps. Nasal stuffiness. Wearing an abdominal binder and leg straps for the urine drainage bag. Indwelling catheter bag has had no new urine for 3 hours.

The nurse has reviewed the Nurses' Notes from Week 5 1400 and the Vital Signs from Week 5 1400.

Place the nurse's actions in the order in which they should be performed. Arrange from first to last.

UNORDERED OPTIONS	YOUR ORDERED STEPS
If the BP stays high after the bladder is emptied, notify the provider	
Sit the client upright and lower his legs	
If urine does not drain, replace the catheter using lidocaine jelly	
Check the catheter tubing for kinks or blockage	
Loosen the abdominal binder and the leg	

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NURS 1017 Unit 7 Case Study 5, screen 5 (answer key, score 1/1)

Practice NCLEX RN Test

Question 5 of 6

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Case Study Screen 5 of 6

The nurse in the emergency department is caring for a 22-year-old male client who dove into shallow water at a lake.

Nurses' Notes | Vital Signs | Diagnostic Results | Laboratory Results | Provider Orders

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

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The nurse has reviewed the Nurses' Notes from Week 5 1400 and the Vital Signs from Week 5 1400.

Place the nurse's actions in the order in which they should be performed. Arrange from first to last.

UNORDERED OPTIONS	YOUR ORDERED STEPS
	1. Sit the client upright and lower his legs
	2. Loosen the abdominal binder and the leg straps
	3. Check the catheter tubing for kinks or blockage
	4. If urine does not drain, replace the catheter using lidocaine jelly
	5. If the BP stays high after the bladder is

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NOT COMPLETE

Case Study Screen 6 of 6

The nurse in the emergency department is caring for a 22-year-old male client who dove into shallow water at a lake.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Laboratory Results** | **Provider Orders**

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

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Week 5 1420: Tubing kinked under his thigh; straightened, 900 mL of urine drained. BP 128/76 mm Hg at 1415, 104/64 mm Hg at 1425. Headache resolving.

Week 10 1000: Discharge planning. The nurse reviews teaching with the client and his brother, who will help with his care at home.

The nurse has reviewed the Nurses' Notes from Week 5 1420 and Week 10 1000.

➤ For each statement, click to specify whether it indicates an **understanding** of the teaching or a **need for further teaching**.

Statement	Understanding	Needs further teaching
Client: "If I get a pounding headache and my face flushes, I'll sit up right away and check my catheter."	○	○
Brother: "When his blood pressure goes up, he should lie down flat."	○	○
Client: "Autonomic dysreflexia only happens in the first few months after an injury."	○	○
Client: "I'll carry my autonomic dysreflexia wallet card and show it to staff if I go to emergency."	○	○
Client: "I'll do my bowel program at the same time every day."	○	○
Brother: "Since he can't feel his legs, we don't need to check his skin."	○	○
Brother: "Next summer, everyone will check the water depth and go in feet first the first time."	○	○

Note: Each row must have only 1 response option selected.

Submit

NURS 1017 Unit 7 Case Study 5, screen 6 (answer key, score 7/7)

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse in the emergency department is caring for a 22-year-old male client who dove into shallow water at a lake.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Laboratory Results** | **Provider Orders**

Day 1 1430: Dove headfirst off a dock into about 1 metre of water at 1340 and struck his head on the bottom. Friends pulled him out of the water, keeping his head in line with his body. Arrived by ambulance with a cervical collar and spinal motion restriction. Alert and oriented × 4. States, "I can't feel or move my legs." Can shrug his shoulders and bend his elbows; cannot straighten his elbows or move his fingers. No sensation below the clavicles or in the arms and hands except the outer arms and thumbs. Legs flaccid with no reflexes; no anal sphincter tone. Breathing is diaphragmatic: the abdomen rises while the chest barely moves; speaks in short sentences; weak cough. Skin warm, dry and pink below the injury. 3-cm abrasion on the forehead.

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Statement	Understanding	Needs further teaching
Client: "If I get a pounding headache and my face flushes, I'll sit up right away and check my catheter."	●	○
Brother: "When his blood pressure goes up, he should lie down flat."	○	●
Client: "Autonomic dysreflexia only happens in the first few months after an injury."	○	●
Client: "I'll carry my autonomic dysreflexia wallet card and show it to staff if I go to emergency."	●	○
Client: "I'll do my bowel program at the same time every day."	●	○
Brother: "Since he can't feel his legs, we don't need to check his skin."	○	●
Brother: "Next summer, everyone will check the water depth and go in feet first the first time."	●	○

Note: Each row must have only 1 response option selected.

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NURS 1017 Unit 7 Case Study 6

76-year-old male client with Parkinson disease taking apixaban who develops delirium from a chronic subdural hematoma weeks after a fall: Parkinson disease vs ALS and MS findings, delirium vs dementia and the types of dementia, bridging-vein bleeding, delirium-safe care, levodopa withdrawal (parkinsonism-hyperpyrexia syndrome), and discharge teaching.

Readiness problems: none

NURS 1017 Unit 7 Case Study 6, screen 1

Practice NCLEX RN Test

Question 1 of 6

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NOT COMPLETE

Case Study Screen 1 of 6

The nurse in the emergency department is caring for a 76-year-old male client with Parkinson disease brought in by his wife.

Nurses' Notes | **History and Physical** | **Vital Signs**

Day 1 1400: Wife states that over the last 10 days he has become increasingly confused and sleepy and has had daily headaches. "Some hours he's his usual self, and other times he doesn't know where he is." Up at night, pulling at his clothes. Four weeks ago he fell in the kitchen and hit his head on a cupboard door, without losing consciousness. Before this he managed his own medications and finances. Assessment: drowsy, easily distracted, cannot name the days of the week backward; oriented to person only. Resting tremor of the right hand ("pill-rolling") that stops when he reaches for a cup; cogwheel rigidity in both arms; slow, small movements; shuffling gait with reduced arm swing (uses a walker); masklike face; soft, monotone voice. New mild weakness of the left arm with a downward drift. No muscle twitching or wasting. Last bowel movement 4 days ago.

The nurse reviews the client's history and assessment. Which of the following findings would be consistent with **Parkinson disease**? **Select all that apply.**

- [ ] 1. New weakness of the left arm with a downward drift
- [ ] 2. Inability to name the days of the week backward
- [ ] 3. Muscle twitching (fasciculations) and wasting
- [ ] 4. Masklike face and a soft, monotone voice
- [ ] 5. Resting tremor of the hand that stops with purposeful movement
- [ ] 6. Shuffling gait with reduced arm swing
- [ ] 7. A drop in BP from 138/78 mm Hg lying to 108/66 mm Hg standing
- [ ] 8. Cogwheel rigidity
- [ ] 9. Slow, small movements (bradykinesia)
- [ ] 10. Tremor that worsens as the hand approaches a target

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NURS 1017 Unit 7 Case Study 6, screen 1 (answer key, score 6/6)

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse in the emergency department is caring for a 76-year-old male client with Parkinson disease brought in by his wife.

Nurses' Notes | **History and Physical** | **Vital Signs**

Day 1 1400: Wife states that over the last 10 days he has become increasingly confused and sleepy and has had daily headaches. "Some hours he's his usual self, and other times he doesn't know where he is." Up at night, pulling at his clothes. Four weeks ago he fell in the kitchen and hit his head on a cupboard door, without losing consciousness. Before this he managed his own medications and finances. Assessment: drowsy, easily distracted, cannot name the days of the week backward; oriented to person only. Resting tremor of the right hand ("pill-rolling") that stops when he reaches for a cup; cogwheel rigidity in both arms; slow, small movements; shuffling gait with reduced arm swing (uses a walker); masklike face; soft, monotone voice. New mild weakness of the left arm with a downward drift. No muscle twitching or wasting. Last bowel movement 4 days ago.

The nurse reviews the client's history and assessment. Which of the following findings would be consistent with **Parkinson disease**? **Select all that apply.**

- [ ] 1. New weakness of the left arm with a downward drift
- [ ] 2. Inability to name the days of the week backward
- [ ] 3. Muscle twitching (fasciculations) and wasting
- [x] 4. Masklike face and a soft, monotone voice
- [x] 5. Resting tremor of the hand that stops with purposeful movement
- [x] 6. Shuffling gait with reduced arm swing
- [x] 7. A drop in BP from 138/78 mm Hg lying to 108/66 mm Hg standing
- [x] 8. Cogwheel rigidity
- [x] 9. Slow, small movements (bradykinesia)
- [ ] 10. Tremor that worsens as the hand approaches a target

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NURS 1017 Unit 7 Case Study 6, screen 2

Practice NCLEX RN Test

Question 2 of 6

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Case Study Screen 2 of 6

The nurse in the emergency department is caring for a 76-year-old male client with Parkinson disease brought in by his wife.

Nurses' Notes | **History and Physical** | **Vital Signs**

Day 1 1400: Wife states that over the last 10 days he has become increasingly confused and sleepy and has had daily headaches. "Some hours he's his usual self, and other times he doesn't know where he is." Up at night, pulling at his clothes. Four weeks ago he fell in the kitchen and hit his head on a cupboard door, without losing consciousness. Before this he managed his own medications and finances. Assessment: drowsy, easily distracted, cannot name the days of the week backward; oriented to person only. Resting tremor of the right hand ("pill-rolling") that stops when he reaches for a cup; cogwheel rigidity in both arms; slow, small movements; shuffling gait with reduced arm swing (uses a walker); masklike face; soft, monotone voice. New mild weakness of the left arm with a downward drift. No muscle twitching or wasting. Last bowel movement 4 days ago.

➤ The client's wife asks whether her husband is "getting dementia." For each feature, click to specify whether it is more consistent with **delirium** or **dementia** (major neurocognitive disorder).

Feature	Delirium	Dementia
Develops over hours to days	○	○
Gradual, progressive decline over months to years	○	○
Inattention, such as being unable to name the days of the week backward	○	○
Alert, with attention preserved until the late stages	○	○
Level of consciousness and confusion fluctuate during the day	○	○
Caused by the progressive loss of neurons, as in Alzheimer disease	○	○
Often reversible when the underlying cause is treated	○	○

Note: Each row must have only 1 response option selected.

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NURS 1017 Unit 7 Case Study 6, screen 2 (answer key, score 7/7)

Practice NCLEX RN Test

Question 2 of 6

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Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in the emergency department is caring for a 76-year-old male client with Parkinson disease brought in by his wife.

Nurses' Notes | **History and Physical** | **Vital Signs**

Day 1 1400: Wife states that over the last 10 days he has become increasingly confused and sleepy and has had daily headaches. "Some hours he's his usual self, and other times he doesn't know where he is." Up at night, pulling at his clothes. Four weeks ago he fell in the kitchen and hit his head on a cupboard door, without losing consciousness. Before this he managed his own medications and finances. Assessment: drowsy, easily distracted, cannot name the days of the week backward; oriented to person only. Resting tremor of the right hand ("pill-rolling") that stops when he reaches for a cup; cogwheel rigidity in both arms; slow, small movements; shuffling gait with reduced arm swing (uses a walker); masklike face; soft, monotone voice. New mild weakness of the left arm with a downward drift. No muscle twitching or wasting. Last bowel movement 4 days ago.

➤ The client's wife asks whether her husband is "getting dementia." For each feature, click to specify whether it is more consistent with **delirium** or **dementia** (major neurocognitive disorder).

Feature	Delirium	Dementia
Develops over hours to days	●	○
Gradual, progressive decline over months to years	○	●
Inattention, such as being unable to name the days of the week backward	●	○
Alert, with attention preserved until the late stages	○	●
Level of consciousness and confusion fluctuate during the day	●	○
Caused by the progressive loss of neurons, as in Alzheimer disease	○	●
Often reversible when the underlying cause is treated	●	○

Note: Each row must have only 1 response option selected.

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NURS 1017 Unit 7 Case Study 6, screen 3

Practice NCLEX RN Test

Question 3 of 6

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Case Study Screen 3 of 6

The nurse in the emergency department is caring for a 76-year-old male client with Parkinson disease brought in by his wife.

Nurses' Notes | History and Physical | Vital Signs | Diagnostic Results | Laboratory Results

Day 1 1400: Wife states that over the last 10 days he has become increasingly confused and sleepy and has had daily headaches. "Some hours he's his usual self, and other times he doesn't know where he is." Up at night, pulling at his clothes. Four weeks ago he fell in the kitchen and hit his head on a cupboard door, without losing consciousness. Before this he managed his own medications and finances. Assessment: drowsy, easily distracted, cannot name the days of the week backward; oriented to person only. Resting tremor of the right hand ("pill-rolling") that stops when he reaches for a cup; cogwheel rigidity in both arms; slow, small movements; shuffling gait with reduced arm swing (uses a walker); masklike face; soft, monotone voice. New mild weakness of the left arm with a downward drift. No muscle twitching or wasting. Last bowel movement 4 days ago.

The nurse has reviewed the Diagnostic Results and the Laboratory Results.

Complete the following sentences by choosing from the lists of options.

The client's acute confusion is most likely Select... caused by Select... . This type of bleed comes from Select... , and his risk was increased by Select... .

Submit

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NURS 1017 Unit 7 Case Study 6, screen 3 (answer key, score 4/4)

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in the emergency department is caring for a 76-year-old male client with Parkinson disease brought in by his wife.

Nurses' Notes | History and Physical | Vital Signs | Diagnostic Results | Laboratory Results

Day 1 1400: Wife states that over the last 10 days he has become increasingly confused and sleepy and has had daily headaches. "Some hours he's his usual self, and other times he doesn't know where he is." Up at night, pulling at his clothes. Four weeks ago he fell in the kitchen and hit his head on a cupboard door, without losing consciousness. Before this he managed his own medications and finances. Assessment: drowsy, easily distracted, cannot name the days of the week backward; oriented to person only. Resting tremor of the right hand ("pill-rolling") that stops when he reaches for a cup; cogwheel rigidity in both arms; slow, small movements; shuffling gait with reduced arm swing (uses a walker); masklike face; soft, monotone voice. New mild weakness of the left arm with a downward drift. No muscle twitching or wasting. Last bowel movement 4 days ago.

The nurse has reviewed the Diagnostic Results and the Laboratory Results.

➤ Complete the following sentences by choosing from the lists of options.

The client's acute confusion is most likely [delirium] caused by [a chronic subdural hematoma]. This type of bleed comes from [torn bridging veins], and his risk was increased by [brain atrophy, frequent falls, and anticoagulant therapy].

Submit

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NURS 1017 Unit 7 Case Study 6, screen 4

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department is caring for a 76-year-old male client with Parkinson disease brought in by his wife.

Nurses' Notes | History and Physical | Vital Signs | Diagnostic Results | Laboratory Results | Provider Orders

Day 1 1400: Wife states that over the last 10 days he has become increasingly confused and sleepy and has had daily headaches. "Some hours he's his usual self, and other times he doesn't know where he is." Up at night, pulling at his clothes. Four weeks ago he fell in the kitchen and hit his head on a cupboard door, without losing consciousness. Before this he managed his own medications and finances. Assessment: drowsy, easily distracted, cannot name the days of the week backward; oriented to person only. Resting tremor of the right hand ("pill-rolling") that stops when he reaches for a cup; cogwheel rigidity in both arms; slow, small movements; shuffling gait with reduced arm swing (uses a walker); masklike face; soft, monotone voice. New mild weakness of the left arm with a downward drift. No muscle twitching or wasting. Last bowel movement 4 days ago.

The nurse has reviewed the Provider Orders.

➤ For each intervention, click to specify whether it is **indicated** or **contraindicated** for the client.

Intervention	Indicated	Contraindicated
Give the carbidopa-levodopa at the exact scheduled times, including while he is NPO	○	○
Give the PRN haloperidol if he becomes agitated	○	○
Reorient him with a clock and calendar, his glasses and hearing aids, and his wife's presence	○	○
Apply wrist restraints at night so he does not get out of bed	○	○
Have him sit and dangle his legs before standing, with assistance	○	○
Keep the room brightly lit and the television on all night to keep him oriented	○	○
Perform the ordered neurological checks, including pupils and arm strength	○	○

Note: Each row must have only 1 response option selected.

Submit

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NURS 1017 Unit 7 Case Study 6, screen 4 (answer key, score 7/7)

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in the emergency department is caring for a 76-year-old male client with Parkinson disease brought in by his wife.

Nurses' Notes | History and Physical | Vital Signs | Diagnostic Results | Laboratory Results | Provider Orders

Day 1 1400: Wife states that over the last 10 days he has become increasingly confused and sleepy and has had daily headaches. "Some hours he's his usual self, and other times he doesn't know where he is." Up at night, pulling at his clothes. Four weeks ago he fell in the kitchen and hit his head on a cupboard door, without losing consciousness. Before this he managed his own medications and finances. Assessment: drowsy, easily distracted, cannot name the days of the week backward; oriented to person only. Resting tremor of the right hand ("pill-rolling") that stops when he reaches for a cup; cogwheel rigidity in both arms; slow, small movements; shuffling gait with reduced arm swing (uses a walker); masklike face; soft, monotone voice. New mild weakness of the left arm with a downward drift. No muscle twitching or wasting. Last bowel movement 4 days ago.

The nurse has reviewed the Provider Orders.

➤ For each intervention, click to specify whether it is **indicated** or **contraindicated** for the client.

Intervention	Indicated	Contraindicated
Give the carbidopa-levodopa at the exact scheduled times, including while he is NPO	●	○
Give the PRN haloperidol if he becomes agitated	○	●
Reorient him with a clock and calendar, his glasses and hearing aids, and his wife's presence	●	○
Apply wrist restraints at night so he does not get out of bed	○	●
Have him sit and dangle his legs before standing, with assistance	●	○
Keep the room brightly lit and the television on all night to keep him oriented	○	●
Perform the ordered neurological checks, including pupils and arm strength	●	○

Note: Each row must have only 1 response option selected.

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Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

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Case Study Screen 5 of 6

The nurse in the emergency department is caring for a 76-year-old male client with Parkinson disease brought in by his wife.

Nurses' Notes | History and Physical | Vital Signs | Diagnostic Results | Laboratory Results | Provider Orders

Day 1 1400: Wife states that over the last 10 days he has become increasingly confused and sleepy and has had daily headaches. "Some hours he's his usual self, and other times he doesn't know where he is." Up at night, pulling at his clothes. Four weeks ago he fell in the kitchen and hit his head on a cupboard door, without losing consciousness. Before this he managed his own medications and finances. Assessment: drowsy, easily distracted, cannot name the days of the week backward; oriented to person only. Resting tremor of the right hand ("pill-rolling") that stops when he reaches for a cup; cogwheel rigidity in both arms; slow, small movements; shuffling gait with reduced arm swing (uses a walker); masklike face; soft, monotone voice. New mild weakness of the left arm with a downward drift. No muscle twitching or wasting. Last bowel movement 4 days ago.

Day 2 1400: Surgery postponed to 1600 because of an emergency case. The 0700 and 1100 carbidopa-levodopa doses were not given because the client was NPO. Client now rigid throughout and unable to turn in bed or swallow his saliva well; more confused; skin hot and sweaty. Pupils equal and reactive; left arm drift unchanged.

The nurse has reviewed the Nurses' Notes from Day 2 1400, the Vital Signs from Day 2 1400 and the Laboratory Results from Day 2 1415.

Which actions should the nurse take? **Select all that apply.**

- [ ] 1. Ask the provider how to give the carbidopa-levodopa now (for example, by nasogastric tube)
- [ ] 2. Give the PRN haloperidol for the increased confusion
- [ ] 3. Apply cooling measures
- [ ] 4. Encourage him to drink water to cool down
- [ ] 5. Monitor urine output and colour
- [ ] 6. Notify the provider of the missed doses and the client's condition
- [ ] 7. Continue to hold the carbidopa-levodopa until after surgery

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NURS 1017 Unit 7 Case Study 6, screen 5 (answer key, score 4/4)

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse in the emergency department is caring for a 76-year-old male client with Parkinson disease brought in by his wife.

Nurses' Notes | History and Physical | Vital Signs | Diagnostic Results | Laboratory Results | Provider Orders

Day 1 1400: Wife states that over the last 10 days he has become increasingly confused and sleepy and has had daily headaches. "Some hours he's his usual self, and other times he doesn't know where he is." Up at night, pulling at his clothes. Four weeks ago he fell in the kitchen and hit his head on a cupboard door, without losing consciousness. Before this he managed his own medications and finances. Assessment: drowsy, easily distracted, cannot name the days of the week backward; oriented to person only. Resting tremor of the right hand ("pill-rolling") that stops when he reaches for a cup; cogwheel rigidity in both arms; slow, small movements; shuffling gait with reduced arm swing (uses a walker); masklike face; soft, monotone voice. New mild weakness of the left arm with a downward drift. No muscle twitching or wasting. Last bowel movement 4 days ago.

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NURS 1017 Unit 7 Case Study 6, screen 6

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse in the emergency department is caring for a 76-year-old male client with Parkinson disease brought in by his wife.

Nurses' Notes | History and Physical | Vital Signs | Diagnostic Results | Laboratory Results | Provider Orders

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Day 3 0900: Levodopa restarted by NG tube at 1530 on Day 2; rigidity and fever settled overnight. Burr-hole drainage of the subdural hematoma done at 0730 today. CK falling.

Day 8 1000: Alert, oriented, attentive (names the months backward). Headache gone; left arm strength equal to the right. Taking carbidopa-levodopa on time by mouth. Discharge home tomorrow with a home care referral. Apixaban to stay on hold until the neurosurgery follow-up. The nurse reviews the discharge teaching with the client and his wife.

The nurse has reviewed the Nurses' Notes from Day 3 0900 and Day 8 1000 and the Vital Signs from Day 8 1000.

➤ For each statement, click to specify whether it indicates an **understanding** of the teaching or a **need for further teaching**.

Statement	Understanding	Needs further teaching
Wife: "His confusion cleared once the blood was drained, so it was delirium, not dementia."	○	○
Client: "I'll take my levodopa with my high-protein breakfast to settle my stomach."	○	○
Wife: "If he gets a new headache or becomes sleepy or confused again, I'll take him to emergency."	○	○
Client: "Now that my head is better, I can restart my apixaban tonight."	○	○
Wife: "His mother had Alzheimer disease, so his confusion means he has it too."	○	○
Wife: "I'll remove the throw rugs and keep a night light on."	○	○
Wife: "If he is in hospital again, I'll ask that his Parkinson pills be given on time."	○	○

Note: Each row must have only 1 response option selected.

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NURS 1017 Unit 7 Case Study 6, screen 6 (answer key, score 7/7)

Practice NCLEX RN Test

Question 6 of 6

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Question Navigator

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Case Study Screen 6 of 6

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Client: "I'll take my levodopa with my high-protein breakfast to settle my stomach."	○	●
Wife: "If he gets a new headache or becomes sleepy or confused again, I'll take him to emergency."	●	○
Client: "Now that my head is better, I can restart my apixaban tonight."	○	●
Wife: "His mother had Alzheimer disease, so his confusion means he has it too."	○	●
Wife: "I'll remove the throw rugs and keep a night light on."	●	○
Wife: "If he is in hospital again, I'll ask that his Parkinson pills be given on time."	●	○

Note: Each row must have only 1 response option selected.

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